

SPARTAN COACHING

HOSPICE SALES EXCELLENCE TRAINING

Medicare/Medicaid Hospice Regulations

Compliance Framework & Referral Optimization

THE FOUR FEDERAL ELIGIBILITY CRITERIA (42 CFR 418.24)

All FOUR must be present. Missing even one disqualifies the patient.

CRITERION 1: Medicare Part A OR Medicaid Eligibility

Patient must have active Medicare Part A or Medicaid. Self-pay only = no hospice eligibility.

CRITERION 2: Physician Certification of Terminal Illness

Two physicians (initial cert) or one + NP (recert) certify terminal illness: 6-month-or-less prognosis. Must expect death, not treatable to cure.

CRITERION 3: Informed Consent via Medicare Election

Patient or representative signs CMS-1525-02 form, understanding they elect hospice and forgo curative treatment (except unrelated conditions).

CRITERION 4: Established Plan of Care

Hospice develops comprehensive care plan addressing pain, symptoms, psychosocial/spiritual needs. Plan documented, reviewed by physician.

DISEASE-SPECIFIC GUIDELINES (Clinical Indicators)

TERMINAL CANCER (Most Common)

Metastatic disease OR locally advanced with systemic symptoms AND functional decline (ECOG 3-4). Regardless of treatment history.

COPD (Emphysema/Chronic Bronchitis)

FEV1 <25% OR resting oxygen sat <88% OR hypercapnia (CO2 >50) OR 2+ hospitalizations in 12 months despite optimal therapy.

HEART FAILURE

NYHA Class IV OR EF <20% OR repeated hospitalizations despite optimal therapy OR inotrope dependence OR physician estimate <6-month survival.

DEMENTIA (Rapidly Growing Non-Cancer)

REISMAN Stage 7C+ (minimal speech, cannot walk/sit, requires total care) AND 12-month ADL decline AND unable to maintain nutrition (weight loss, difficulty swallowing).

RENAL DISEASE

Stage 5 (GFR <15, NOT continuing dialysis) with creatinine >2.5 AND declining function AND medical complications (hypertension, pericarditis, infections).

THE OPTIMAL 4-STEP REFERRAL PROCESS

Step 1: EARLY IDENTIFICATION (Days 1-7)

Care team flags patient meeting criteria. Document in care plan. Goal: Identify within 7 days, not at discharge planning.

Step 2: PHYSICIAN DISCUSSION (Days 3-10)

Attending physician meets patient/family. Discusses prognosis, options, hospice as option. Physician certifies if appropriate. Target: <7 days from identification to certification.

Step 3: FAMILY INFORMED CONSENT (Days 7-14)

Coordinator explains hospice benefits, obtains informed consent, explains differences from curative treatment. Family signs CMS-1525-02. Goal: Same-day/next-day after physician discussion.

Step 4: HOSPICE ADMISSION & CARE PLANNING (Days 10-21)

Hospice completes intake, develops individualized care plan. Services begin immediately. Physician reviews and signs plan within 48 hours.

